

CLINICAL INTELLIGENCE TREND REPORT

RED: Provider Review Recommended

Patient ID: S (Chair)Period: March 30 to June 28, 2024Report Date: June 29, 2024Coverage: 1857/2184h (85%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm

Patient clinical status over 91 days from Mar 30 to Jun 28

HR

Breathing

Event	Period
Elevated Heart Rate	Jun 26 to Jun 28 (3d)
Elevated Breathing	Apr 03 to Apr 03 (1d)
Elevated Breathing	Apr 10 to Apr 10 (1d)
Elevated Breathing	May 12 to May 12 (1d)
Elevated Breathing	Jun 21 to Jun 24 (4d)
High Heart Rate	Jun 27 to Jun 27 (1d)
Low Heart Rate	Jun 24 to Jun 25 (2d)

Last 24 Hours

Last 24 Hours: ELEVATED

30-Day Tier: RED

Vital	Avg	Min	Max
Heart Rate	91 bpm	60 bpm	109 bpm
Breathing	12 brpm	9 brpm	25 brpm

Last 24 hours: two elevated heart rate episodes overnight; one high heart rate episode overnight.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Jun 28	12 AM to 3 AM	8h	3	Elevated Breathing	12	9	25	Check SpO2 and lung sounds; assess for fluid overload or early infection
Jun 28	3 AM to 6 AM	5h	2	Elevated Heart Rate (brief)	97	83	104	Check temp, hydration, pain; review for arrhythmia or infection
Jun 28	12 AM to 3 AM	3h	1	High Heart Rate (brief)	102	91	109	Check temp, hydration, pain; review for arrhythmia or infection

18 episodic events Detected, spanning 42h total. Conditions: Elevated Breathing, Elevated Heart Rate, High Heart Rate, Low Heart Rate. Priority grade: High

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
18	42h	Elevated Heart Rate	<1	85%

Major Findings: Sustained elevated HR (avg 97, peak 109)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Jun 27	8 PM to 3 AM	7h	1	High Heart Rate	102	91	109	Check temp, hydration, pain; review for arrhythmia or infection
Jun 24	2 AM to 3 AM	4h	2	Low Heart Rate	43	40	46	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Apr 03	1 AM to 2 AM	1h	1	Elevated Breathing	26	26	26	Check SpO2 and lung sounds; assess for fluid overload or early infection
Apr 10	1 AM to 2 AM	1h	1	Elevated Breathing	24	19	30	Check SpO2 and lung sounds; assess for fluid overload or early infection
May 12	12 AM to 1 AM	1h	1	Elevated Breathing	24	18	28	Check SpO2 and lung sounds; assess for fluid overload or early infection
Jun 21	5 PM to 6 PM	13h	4	Elevated Breathing	28	15	44	Check SpO2 and lung sounds; assess for fluid overload or early infection

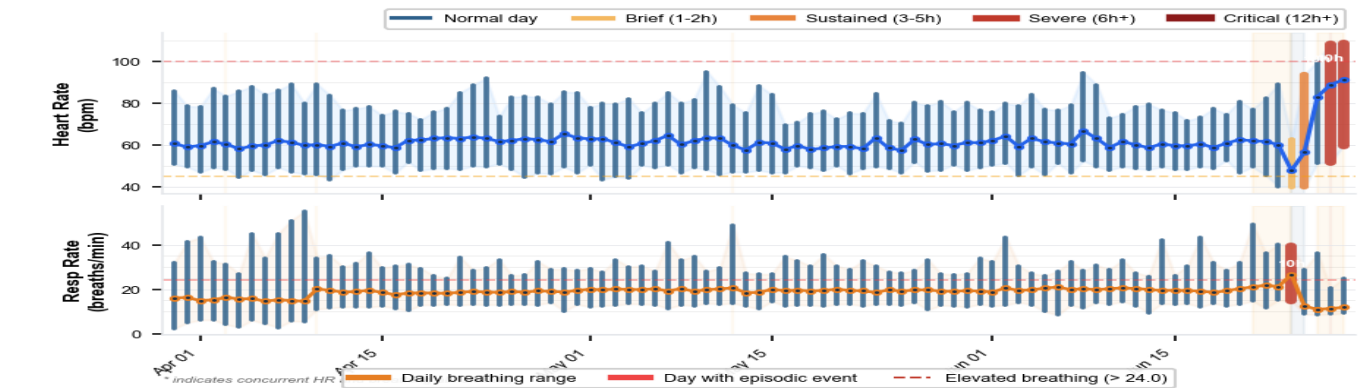
+ 1 additional condition(s) across Elevated Heart Rate; details in monitoring data.

Suggested Clinical Review Actions

- Elevated Heart Rate (avg 97 bpm, peak 106 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Elevated Breathing (avg 28 brpm, peak 44 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Heart Rate (avg 102 bpm, peak 109 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Low Heart Rate (avg 43 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Overall trajectory shows 7 unstable phases with 20 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

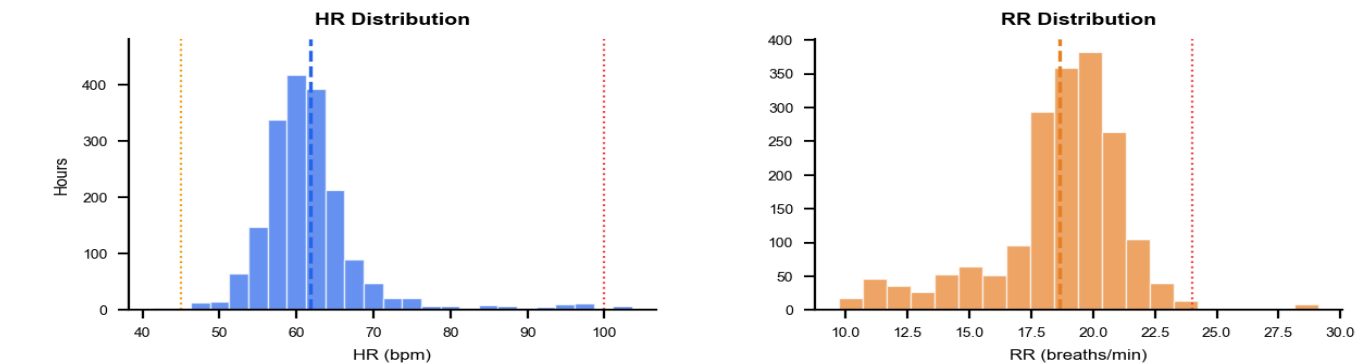
Clinical Pattern Observations:

- 1. Sustained finding: 10h continuous Elevated Breathing on Jun 24.
- 2. Clustered pattern: Episodic events on 4 of 91 days (4%).
- 3. Nocturnal pattern: 6 of 6 eligible episodes during evening or night hours.

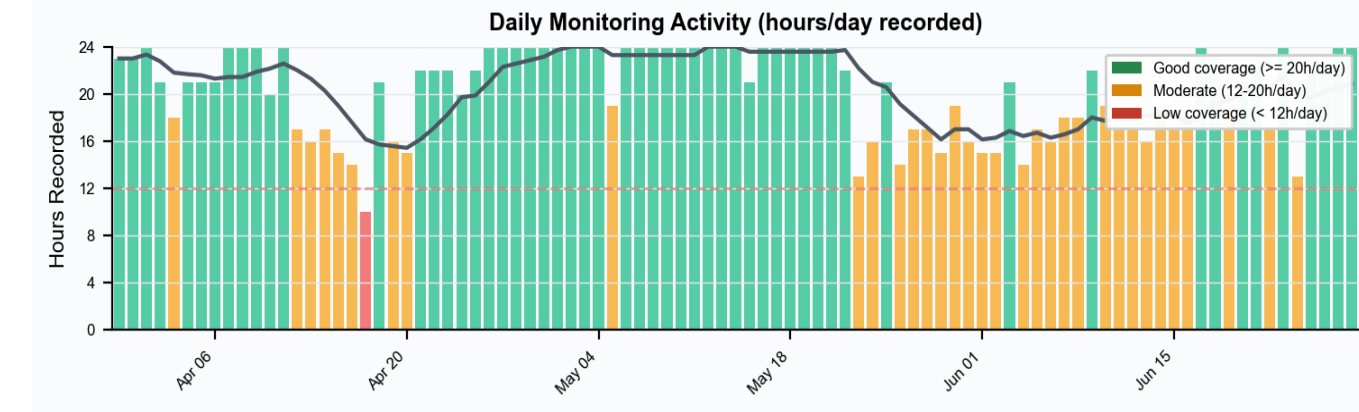


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.